

PREPARED FOR DR RICK NGO • AUGUST 2026

We run the patient contact around a procedure

From the day the case is booked to thirty days after it, so the work you hand the patient actually gets done, and what goes wrong at home reaches your nurse instead of an emergency department.

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How we ensure your list runs and patients get the best outcomes

The same case, twice. Time runs left to right.

	AT BOOKING	2 WEEKS OUT	48 HOURS OUT	DAY OF	DAYS 1 TO 3	WEEKS 1 TO 4
Today status quo	Instructions handed over. Nothing more until pre-admission	Staff start phone rounds. Voicemail, callbacks, repeat attempts	Confirmation calls. Anything found now is too late to fix	Cancellation discovered on arrival. Slot lost	Patient calls the office, or drives to an emergency room	Silence read as recovery. Recall slips
With Aescia	Patient onboards to Aescia, which - educates - prepares - verifies needs	Holds, fasting, escort and cardiology clearances confirmed automatically	Readiness verified. A drop-out is known while the slot can still be refilled	Full list, prepared patients	Symptoms triaged. Urgent items to a nurse the same day	Ensures the discharge plan is followed. Recall tracked and booked
Empty slot cost to refill	TRIVIAL	WORKABLE	HARD	GONE		

Pick your problems and price them →

Interactive. Choose what is real at your centers and it totals as you go. aesciahealth.com/tools/general-surgery/problems

What we believe

01 The failure happens at home. In the days either side, unsupervised, where nobody is looking and nothing is measured. The procedure itself is the safe part.	02 Preparation has to start at booking. Pre-admission sits one to two weeks out, by which time the window to fix anything has closed. Booking is the only moment early enough.	03 The questions are not the product. The value is in the guidance attached to the answer, a human call when someone goes quiet, and a summary in front of whoever reviews it.
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What it does for a surgery center

One engine, different content per procedure. A general surgery day case is a colonoscopy minus the bowel prep.

GASTROENTEROLOGY	GENERAL SURGERY
Protect the slot and the scope - Bowel preparation is one to three days of unsupervised patient work, and it is where most avoidable failure lives - Prep quality checked before the patient travels, not discovered on the table - Cancellations flagged early enough that the slot can still be refilled - Surveillance recall tracked, so the three-year patient comes back	Protect the morning and the week after - Fasting, escort and anticoagulant holds confirmed rather than assumed - Day-of cancellations caught days early, while the list can still be rearranged - Retention, pain and wound worries routed to a nurse instead of an emergency room - The post-operative call burden lifted off your office

One vendor across the list. A multi-specialty center runs a single integration and a single pathway builder, rather than one product per service line.

What I would like from you

The next step is a trial at a surgery center.

- Your read on this as a surgeon,** and any other operators you know who might have thoughts. Where you think it breaks, and what you would want to see before you let it near your list.
- An introduction to the person who makes decisions at your ASCs.** Thirty minutes, no commitment beyond hearing it.
- Which of this is worth money at your centers, and to whom.**

If this moves in Texas, we will be looking at advisory positions.

Live monitoring trial at Royal Prince Alfred Hospital, Sydney · ACTRN12625001425482 · interim readout September 2026

Interactive version: aesciahealth.com/tools/general-surgery/problems · aesciahealth.com · james@aesciahealth.com